

SUPPORT SUPERVISION TOOL FOR SENTINEL SITES ON INTERGRATION - Maturity Staging

SUPPORT SUPERVISION TOOL FOR SENTINEL SITES ON INTERGRATION

This monitoring and mentorship tool is intended to assess progress, check best practices and identify challenges in need of solutions to support the integration of standalone services into the primary health care system in Uganda. It checks implementation of the Ministry of Health RoadMap on improving service delivery in both central and local government health facilities in Uganda.

Select your preferred tool choice

Region, District or Facility



Regional Assesement



District Assesement



Facility Maturity Staging

Region



Acholi



Ankole



Bugisu



Bukedi



Bunyoro



Busoga



Kampala



Karamoja



Kigezi



Lango



NorthCentral



SouthCentral



Teso



Tooro



WestNile

District

Facility

Level of Health Centre



National Regional Referral Hospital



Regional Referral Hospital



Hospital



HCIV



HCIII



HCII



Community

Ownership



GoU



PNFP



PPF

Date of Visit

yyyy-mm-dd

Introduction

Section 1: Mentorship Guide

1. Introduction On February 7, 2025, the Ministry of Health directed all health facility managers to integrate HIV/AIDS, TB, Hepatitis B, Hypertension, and Diabetes services into routine care at all facility levels, marking a shift from vertical to integrated service delivery. Since then, the Ministry has developed integration guidelines, training materials, and conducted capacity-building activities. This mentorship guide has been developed to support mentors in providing ongoing guidance to frontline health workers for the effective implementation of integrated health services.

Purpose of the mentorship guide

To provide a structured framework for implementing mentorship and coaching that supports health facilities in delivering integrated health services effectively, by continuously strengthening the capacity of health workers to improve service delivery and client outcomes.

Objectives of the mentorship

*Build clinical and leadership competencies of health workers to implement integrated services * Strengthen the quality of care * Improve client satisfaction * Enhance teamwork, communication, and accountability*

2. Mentorship Process.

*A. Outputs * Functional multi-disciplinary team established and oriented * Performance improvement plans developed * Quality improvement projects initiated * Client feedback initiative implemented * Facility-level dashboards tracking key performance indicators*

B. Planning

** Conduct a joint orientation or preparatory meetings for mentors at the region/ district * Engage with the facility teams and set a schedule for the mentorship * Set clear expectations for the site-level engagement*

C. Site mentorship process

- Meet the facility in-charge to map out the flow of the mentorship and coordinate the facility team * Conduct the assessment and staging of the facility * Review the implementation progress of the previous action plan * Move around the facility to observe client flow and how integrated services are provided * Review the documentation journal to track the performance of key improvement objectives*

D. Tools and Templates

- Mentorship guide * Assessment tool * Staging tool * Integration practical guide*

3. Sustainability Strategies

- Embed mentorship into facility performance management. * Establish a mentorship pool within the facility or district. * Integrate quality improvement in the mentorship/supervision activities.*

Assessors/Mentors

1

Name

* Organization/ Affiliation

- ☐ Ministry of Health Team
 ☐ Regional Referral Hospital Team
 ☐ District Health Team
 ☐ Facility self assessment

Organization/ Affiliation

Review of previous actions

Check action tracker

Regional Assessment

1. Does the region have an integration committee?

- ☐ Yes
- ☐ No
- ☐ NA

Comment

2. Does the region have a focal person – technical lead for integration at the regional level?

- ☐ Yes
- ☐ No
- ☐ NA

Comment

3. Integration component included in the quarterly regional Health Partner meetings

- ☐ Yes
- ☐ No
- ☐ NA

Comment

4. Integration component included in the Regional performance review meetings

- ☐ Yes
- ☐ No
- ☐ NA

Comment

5. Has the region set up regular (at least Monthly) meetings with the districts to functionalise regional integrated care system platforms

- ☐ Yes
- ☐ No
- ☐ NA

Comment

6. Has the region conducted at least once a quarter support supervision visits to the districts/selected facilities to support integration efforts

- ☐ Yes
- ☐ No
- ☐ NA

Comment

District Assessment

» Standard 1: Develop and implement integration action plans**1. District/City has District/City Strategic Plan with a component of integrated health service delivery***Check the district/city strategic plan - where integration is not included engage DHO and CAO to advocate with the district leadership*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. District/City Health Teams have an Annual Workplan for integrated health service delivery*Check to see the work plan. If not available support the district to come up with key activities for the plan*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

3. 80% of health facilities in the district have annual work plans for integration

Check if the district has a copy of the site plans and enumerate them

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» Standard 2: Share information on integration with political leaders, CSOs and religious leaders to support advocacy and client literacy**1. District has engaged Stakeholders to advocate for integrated service delivery**

check for evidence of the district stakeholder meetings on integration

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. District follows up with the stakeholders to get feedback on how they are supporting integrated service delivery

check reports of the partners or minutes of the stakeholder meetings

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» Standard 3: Conduct support supervision and monitoring the integration of standalone services**1. District has conducted integrated support supervision on integrated service delivery to the health facilities**

Check reports, if no supervision, support the district to plan one

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. District has implemented key actions from the last integrated support supervision visits on integrated service delivery*Check evidence of key actions and status on progress*☐ Yes☐ No☐ NA**Comments / Findings**

3. Is there evidence that the district has addressed any of the integration-related challenges*Check reports*☐ Yes☐ No☐ NA**Comments / Findings**

» Standard 4: Establish and functionalise district integrated care system platforms to strengthen coordination and oversight of lower facilities.**1. Has the district set up regular (at least Monthly) review meetings with the sites to review their integration plans***Check for evidence of the learning meetings*☐ Yes☐ No☐ NA**Comments / Findings**

2. District has documented and disseminated best practices on intergrated service delivery*Check the best practices documentation*☐ Yes☐ No☐ NA**Comments / Findings**

11. PHC Funds have been reallocated to support Integrated delivery of Health Services?*(Probe what the PHC funds are supporting)*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

Facility Maturity Staging

» Domain 1: Leadership and Governance**» » Standard 1: Work Plan for Integration****1. Does the facility have integration work plan for Integrated Health Service Delivery?***Check availability of the action plan What has been done to actively engage in promoting integrated approaches to HIV and broader health systems strengthening?*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Does the facility have a coordinator for integration ?*If the facility has no focal person, engage the facility leader to nominate a focal person. If there is a focal person, check if they have clear roles including coordination and oversight of integration activities, communication and advocacy and monitoring*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

3. Does the Action plan cover all domains for Integration? (Domains include leadership, service delivery, supply chain, HMIS, HRH, laboratory, Financing and QI)*If the facility integration work plan does not cover the different domains of the work plan; come back after the assessment to support the team review the action plan to include areas of needs across the different domains*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

4. Is their evidence of the integration work plan being implemented?

Together with the team, review progress of the action plan to identify and discuss areas that have not been implemented and offer necessary support and guidance

☐ Yes

☐ No

☐ NA

Comments / Findings

» » Standard 2: Facility Leadership Capacity for Integration**1. Has the facility leadership (Incharge and focal person) attended a training or orientation meeting on integration.**

☐ Yes

☐ No

☐ NA

Comments / Findings

2. Have health workers been oriented on the Ministry of Health's integration circular/practical guide?

If not, get/share a copy of the circular and share with the team for reference

☐ Yes

☐ No

☐ NA

Comments / Findings

3. Has the facility leadership held monthly meetings focused on Integration? Check evidence of the last month

Where integration meetings can not be held independently, Integration should be part of the agenda of the facility meetings. The focal person or a designated person should provide regular updates and discuss challenges and support required

☐ Yes

☐ No

☐ NA

Comments / Findings

» » Standard 3: Internal Support Supervision**1. Is there an active monitoring and supervision mechanism in place for the integration process?**

Check if the in-charge regularly conducts monitoring activities - the team should be able to describe how this is implemented.

☐ Yes

☐ No

☐ NA

Comments / Findings

2. Are there supervision activities implemented in the last one month?

Check the plan for these activities and meeting notes

☐ Yes

☐ No

☐ NA

Comments / Findings

3. Is there a report on outcomes of internal support supervision?

What are the best practices that leadership can share on integration experience?

☐ Yes

☐ No

☐ NA

Comments / Findings

» Domain 2: Service Delivery**» » Standard 1: Integrated Chronic and Acute Care established****1. Is the facility providing integrated health services - either Mixed OPD model or Acute OPD & Chronic care clinics?**

If no, probe for reasons why they still have a standalone clinic Why hasn't the facility yet integrated? If yes, specify which clients are attending the integrated chronic care clinic. Does it include all persons regardless of HIV status? What has been done to engage communities of PLHIV and other chronic care

☐ Yes

☐ No

☐ NA

Comments / Findings

2. Does the facility have an intergrated chronic care clinic?

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

3. Does the facility have an intergrated acute care clinic?

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

4. Has the facility put signage to guide patient flow in the integrated clinic spaces?

Move through the clinic to experience the patient flow, identify any areas that need

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

5. Are patients screened for multiple conditions (HIV, TB, Hepatitis, NCDs) at their clinical encounter at OPD/ chronic care clinics?

Check the documentation to confirm screening for multiple conditions. Where there is no evidence of complete screening review the clinic processes wth the team and guide them

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

6. The site implements Integrated Differentiated Service Delivery Models with at least 3-4 chronic care conditions for example that include TB, HIV, DM and HT*check the evidence if service delivery models cover 3-4 chronic conditions*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

7. Are newly diagnosed patients provided appropriate treatment by the same health care workers in the integrated service point?*Sample and check records for the same review period for evidence of care of same day of diagnosis and by same provider.*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 2: Availability of Clinical Guidelines on Integration**1. Are the Uganda Clinical Guidelines/Job Aids/SOPS available in clinics?***These tools may be printed copies, soft copies or online copies for reference*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Do all staff who provide integrated services have access to the Uganda clinical Guidelines/Job Aids/SoPs*Staff should know and have access to materials for reference*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 3: Referral Systems**1. Is there a system for referral of patients within the health facility for appropriate further or advanced management?**

Check for SoPs - where there are no SoPs, guide the team to develop a workable SoPs within their context

☐ Yes

☐ No

☐ NA

Comments / Findings

2. Is there a system for referral of patients outside the health facility for appropriate further or advanced management?

Check linkage to appropriate clinical service after diagnosis within the same facility or outside the facility. Check for referral forms, referral register, documented procedure

☐ Yes

☐ No

☐ NA

Comments / Findings

» » Standard 4: Patient Education**1. Does the facility have integrated IEC Materials**

check what materials are available and if in use for patient education

☐ Yes

☐ No

☐ NA

Comments / Findings

2. Does the facility disseminate integrated IEC material

☐ Yes

☐ No

☐ NA

Comments / Findings

3. Does the conduct integrated health education and promotion programs on service delivery

check the schedule and if integrated health services is covered among the topics

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 5: Patient Follow up**1. Does the facility have a functional-integrated patient appointment system?**

Probe for integrated registration of appointments to track scheduled attendance.

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Does the facility have a functional-integrated patient appointment reminder system?

Probe for integrated pre-appointment reminders, integrated community follow-up. Check for leverages on immunisation platforms and DSDM platforms.

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

3. Are patient support groups established for chronic disease management?

Where only disease-based groups exist, guide the team to make them inclusive. Share the MOH NCD club Memo for reference

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

4. Does the facility conduct routine follow-up of chronic disease patients?

Check chronic patient retention, SoP for managing follow-up services and evidence of successful follow up

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 6: Integrated Maternal Health Services**1. Does the facility deliver Integrated ANC including Elimination of Vertical Transmission services?**

Check if women receive screening and treatment for HIV, Hepatitis B and Syphilis

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Does the facility provide respectful obstetric care

Check feed back from women if they received dignified delivery

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

3. Does the facility deliver integrated Post Natal Care services for mothers and babies?

Check if women receive postnatal retesting and prophylaxis, Check if babies receive screening and prophylaxis, Check if

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

4. Does the facility follow up Mother-Baby Pairs*Check if the facility follows mother baby pairs*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 7: Integrated Pediatric and Adolescent Care**1. Does the facility deliver integrated pediatric care***Check if children are screened, receive prophylaxis and management of conditions in the same visit*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Does the facility deliver integrated adolescent care services*Check if Adolescents are screened, receive prophylaxis and management of conditions in the same visit*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

3. Does the facility have designated space/days for adolescent care*Check if the facility has designated space/clinic days for adolescent care*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

4. Does the facility provide DSD for children and adolescents with chronic conditions*Check if the facility has DSD models for chronic conditions*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

5. Does the facility monitor children and adolescents*check if children and adolescents have updated monitoring results(Viral Load, HB1AC, HB)*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 8: Infection Prevention and Control**1.Does the facility have IPC protocols in place***Check if the IPC protocol is displayed at clinic spaces*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2.Does the facility have IPC facilities in place*Check if the facility has Hand washing, PPE, Waste management facilities*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

3. Do health workers observe standard precautions in patient care

Check if the health workers conduct risk assesment, hand, respiratory hygiene, waste management and decontamination

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

4. Does the facility have an Isolation space and protocol

Check if the facility has an isolation protocol and facility

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 9: Emergency Care**1. Does the facility have an emergency care protocol displayed**

Check if UTAT is displayed

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Does the facility triage patients for emergency signs

Check if triage is conducted and patients have vital signs recorded

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

3. Are health workers trained in Emergency Care

Check if health workers have completed the BLS, ALS training

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

4. Are emergencies managed appropriately

Check if the MoH Medical Emergency Checklists, ambulance transfer protocols are completed

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» Domain 3: Supply Chain and Medicines Management**» » Standard 1: Integrated Quantification and Ordering of EMHS****1. Is there a joint quantification process for all essential medicines?**

Do they work in an integrated manner in forecasting, ordering etc?

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Does the facility jointly prepare orders for all essential commodities?

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 2: Integrated Stock & Inventory Management

1. Are all commodities and supplies recieved through the main store?

Are orders prepared in an integrated manner? Does the ordering involve a multidisciplinary team? Check last order to confirm integration of lab and pharmacy commodities and order submission dates- are they the same?

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Does the facility monitor stock status for all commodities

Probe for a one stock card policy for all items

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 3: Integrated OPD dispensing Points

1. Does the facility have integrated OPD Dispensing points

Probe for multiple stores & dispensing points, who manages the different stores, probe for designation of commodities to the different stores

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Does the facility accord privacy to clients during the dispensing of medicines

Check the dispensing points and observe for privacy. Where there are privacy concerns, discuss and guide the team to appropriately address this

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 4: Integrated Medicines Accountability Processes**1. Does the facility have an active multidisciplinary Medicines and Therapeutic Drug Committee that oversees Appropriate medicines use?***Check minutes and composition of the MTC?*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Does the incharge and MTC conduct quarterly medicine audits for tracer commodities

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 5: Integrated Medicine Information Systems**1. Is the facility updating the Integrated Dispensing Log?***Check availability of tracer commodities in the last 1 month - TLD90, RHZE, Artemether Lumefantrine, Metformin 500, Amlodipine 5, TDF, Determine, DMPA. Has the facility faced major stockout of EMHS in the last 1 month?*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Does the facility update essential Medicine data systems

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» Domain 4:Health Management Information System (HMIS)**» » Standard 1: Utilisation of Key HMIS Tools (Electronic/paper based)****1. Are standardized HMIS tools available and used correctly?***Does the facility submit routine HMIS reports on time?*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Are data quality assurance mechanisms in place to ensure the completeness and accuracy of data?*Check DQA-related reports and actions to address the DQA issues*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 2: Integrated Data Capture in HMIS tools**1. Is the facility updating the electronic or manual HMIS system for all conditions?***Probe for (Sample charts and registers)/ electronic records*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 3: Integrated DHIS Reporting**1. Is the facility timely submitting HMIS 105 reports for all disease conditions?**

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 4: Integrated Utilisation of Data**1. Does the facility have scheduled data review meetings to discuss the performance of key Indicators**

What adaptations have been made to conduct integrated (and not disease specific) performance reviews?

☐ Yes

☐ No

☐ NA

Comments / Findings

2. Does the facility have key actions from the performance review meeting on integrated health service delivery?

check the key actions

☐ Yes

☐ No

☐ NA

Comments / Findings

» Domain 5: Human Resources for Health**» » Standard 1: Availability of Staff dedicated to Integrated Delivery of Services****1. Has the facility identified critical cadres to work in the integrated services**

Check attendance of staff to duty and staffing norms versus the new structure. Probe cadres

☐ Yes

☐ No

☐ NA

Comments / Findings

2. Is there a staff schedule for the integrated services

Check the schedule to see if all critical cadres are included

☐ Yes

☐ No

☐ NA

Comments / Findings

"3. Are all allocated staff routinely engaged in providing integrated service delivery clinics? "*Probe regular attendance of the staff*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

4. Are tasks distributed among GoU and Partner-supported staff to deliver integrated services at the clinical and psychosocial points of care*Probe for the presence of GoU and Partner staff for clinical and psychosocial care tasks*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 2:Capacity Development for Staff to deliver Integrated Care**1. Have the allocated staff received training on Integrated Health Service Delivery?***Probe how many health workers were working in HIV units and how many in OPD?*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Have the allocated staff received disease specific training for the integrated package of services

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

3 Are there mentorship and coaching programs for continuous capacity building of the team?*Probe when last coaching happened*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

4. Does the facility have an integrated CME schedule?*Check the CME schedule and ask if the last 3 sessions were conducted*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» Domain 6: Laboratory Systems and Diagnostics**» » Standard 1: Availability of Appropriate laboratory Services for Integration****1. Is the laboratory able to provide services for the various conditions under integrated services? At least 80% of expected laboratory and diagnostic services.***Check for the menu of basic tests such as HIV, TB, RBS, Malaria....*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 2: Integrated Laboratory Work Flows (Sample Collection, processing and results)**2. Is the main Laboratory coordinating sample collection and return of results for all units?**

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 3: Integrated result reporting (dispatch and timelines)**1. Is the average turn around time for the Results within acceptable standard**

This is easier where the facility has an electronic system in place. Request to team to run the data for review.

☐ Yes

☐ No

☐ NA

Comments / Findings

» Domain 7: Community Systems and Routine Services**» » Standard 1. Integrated Facility driven community outreaches****1. Does the facility have a schedule for integrated community services?**

If there is no schedule guide the facility to develop one

☐ Yes

☐ No

☐ NA

Comments / Findings

2. Does the facility provide integrated community outreach services?

Check the recently conducted outreach reports.

☐ Yes

☐ No

☐ NA

Comments / Findings

3. Are all community health workers (CHEWS, VHTs, Linkage facilitators, expert clients, peer leaders) providing integrated services

Engage the CHW to provide insights on who they reach with services and what services are provided. Guide on providing integrated services at each encounter and referral where appropriate

☐ Yes

☐ No

☐ NA

Comments / Findings

» » Standard 2. Integrated Community Health Promotion, Prevention and Treatment

1. Do the CHW provide the integrated essential community health package (including case detection, medicine management, patient follow up and referral for Malaria, HIV, TB, NCD, and GBV.) services provided in the community outreaches

Review evidence of multi-disease line lists, tracking outcomes, and community providers. Refer to the national community health strategy.

☐ Yes

☐ No

☐ NA

Comments / Findings

» » Standard 3. Linkage and Referrals (Immunisation, Supplementation, DSD Models)

1. Does this facility have a referral directory for all the support service points e.g. CBOs, CSO, Police, e.t.c for patient support?

Check for the referral directory in view, with range of services and contact information.

☐ Yes

☐ No

☐ NA

Comments / Findings

2. If yes to question above, does the facility have evidence of facility-community and community facility referrals?

Check community facility register and referral forms - if all services are provided

☐ Yes

☐ No

☐ NA

Comments / Findings

3. Does the facility have integrated literacy materials for patient/care taker and community education at facility and community?

Check for hand books and flipcharts

☐ Yes

☐ No

☐ NA

Comments / Findings

» Domain 8: Health Financing

» » Standard 1: Allocation of PHC resources for Integration

1. Have PHC Funds been reallocated to support Integrated delivery of Health Services ?

"Supervisor checks if funds are supporting Human resources for health, Integrated outreaches (including community follow up of chronic care clients), Laboratory sample transportation (fuel costs and hub rider payments), Mobile data to support obtaining results. What has been done to mobilize domestic resources (PHC) to fill gaps created by reductions in development partner support?"

☐ Yes

☐ No

☐ NA

Comments / Findings

» » Standard 2: Allocation of Donor funds to Integration

2. If yes, what are the PHC funds supporting?

"Supervisor checks if funds are supporting Human resources for health, Integrated outreaches (including community follow up of chronic care clients), Laboratory sample transportation (fuel costs and hub rider payments), Mobile data to support obtaining results. What has been done to mobilize domestic resources (PHC) to fill gaps created by reductions in development partner support?"

☐ Yes

☐ No

☐ NA

Comments / Findings

» Domain 9: Quality Improvement

» » Standard 1: Facility QI structure

1. Has facility established a WIT for integrated services?

A WIT is a last functional unit of implementation. An integrated OPD or Chronic care clinic should have a WIT that meets weekly to review their processes of care and patient concerns. A WIT should have a leader and scheduled meetings. Where there is no WIT, support team to form one of at least 5 members but less than 10 representing the unit.

☐ Yes

☐ No

☐ NA

Comments / Findings

2. Is there evidence the integrated clinic WIT meets every week?*check meeting notes for the last 3 weeks*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

3. Is there evidence of an improvement initiative undertaken by the WIT in the integrated clinic.*check documentation journal and meeting notes*

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

4. Does the facility have an up-to-date action plan to implement the integrated services improvement initiatives

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 2: Quality Improvement Projects on Integration**1. Implementing at least 2 integrated services-related QI projects**

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Is their evidence of meeting the improvement target for the QI projects

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 3: Routine Client Satisfaction Feedback**1. Does the integrated services unit have last month's client satisfaction data in the dashboard?**

Check the dashboard.

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Is there evidence that the data on client satisfaction has been utilised to address client concerns?

Check if site has login credentials and able to download data for further analysis. Does the team know their clients biggest concerns? Have they put in place emechanisms to address those concerns?

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

3. Clients of integrated services attain client satisfaction rate at 80% or more?

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

» » Standard 4: Shared Learning for Integration**1. Share performance / progress on integration within exisiting meetings including facility meetings, department meetings**

- ☐ Yes
- ☐ No
- ☐ NA

Comments / Findings

2. Does the facility participate in sharing lessons learned in implementing integrated services with other health facilities☐ Yes☐ No☐ NA**Comments / Findings**

Key Performance Indicators (Moved to an indipennt link)**» Domain 1: Leadership and Governance****» » % of quarterly ISD performance reviews conducted**

Numerator: Number of entities (facility level) that conducted quarterly integrated service delivery performance reviews

Denominator: Total number of quarterly integrated service delivery performance reviews scheduled at the facility level

» » % of recommendations that have been implemented

Numerator: Number of action plan recommendations (generated during the quarterly ISD performance review) implemented

Report and Aide Memoir

Denominator: Total number of actions/recommendations generated during the quarterly integrated service delivery performance review

Report and Aide Memoir

» » Number of support supervisions received in the last quarter

Numerator: Number of integrated support supervision visits received at the facility in the last quarter

Performance Review Issues Log

Denominator: N/A (Count indicator — no denominator)

Performance Review Issues Log

» » Semi-annual work plan implementation rate

Numerator: Number of semi-annual work plan activities for integration implemented

Quarterly Performance Reports

Denominator: Total number of semi-annual work plan activities for integration planned

Quarterly Performance Reports

» Domain 2: Service Delivery

» » % of eligible patients with chronic conditions enrolled in the integrated chronic care clinic

Numerator: Number of eligible patients with chronic conditions enrolled in the integrated chronic care clinic

Chronic care clinic register / DHIS2

Denominator: Total number of eligible patients with chronic conditions (HIV, TB, DM, HT, Mental Health) registered at the facility

» » % of chronic care patients screened for comorbidities (HIV, TB, DM, HT) in the same visit

Numerator: Number of chronic care patients screened for comorbidities (HIV, TB, DM, HT) in the same visit

Integrated register / observation

Denominator: Total number of chronic care patients seen at the facility in the reporting period

» » % of chronic care patients managed for multiple conditions in the same visit

Numerator: Number of chronic care patients managed for multiple conditions in the same visit

Integrated register

Denominator: Total number of chronic care patients presenting with multiple conditions in the reporting period

» » % of eligible chronic care patients enrolled on Differentiated Service Delivery (DSD) models

Numerator: Number of eligible chronic care patients enrolled on Integrated Differentiated Service Delivery (DSD) models (MMD, community groups, fast-track)

DSD register / DHIS2

Denominator: Total number of eligible patients with chronic conditions registered at the facility

» » % of chronic care patients appropriately referred (internal & external)

Numerator: Number of chronic care patients appropriately referred (internal and external) with complete referral documentation

Referral register

Denominator: Total number of chronic care patients who required referral in the reporting period

» » % of chronic care patients tracked through the integrated appointment system

Numerator: Number of chronic care patients registered and tracked through the integrated appointment system

Appointment register / DHIS2

Denominator: Total number of chronic care patients requiring scheduled follow-up at the facility

» » % of mortality audits for chronic conditions conducted as scheduled

Numerator: Number of mortality audits for chronic conditions conducted as scheduled in the reporting period

Mortality audit register

Denominator: Total number of mortality audits for chronic conditions scheduled in the reporting period

» » % of pregnant women attending ANC who received integrated services in the same visit

Numerator: Number of pregnant women attending ANC who received integrated services (HIV testing, malaria prophylaxis, nutrition assessment, NCD screening) in the same visit

ANC register / Integrated register / DHIS2

Denominator: Total number of pregnant women attending ANC in the reporting period

» » % of maternal deaths with a completed mortality audit

Numerator: Number of maternal deaths with a completed mortality audit conducted as scheduled

Denominator: Total number of maternal deaths recorded at the facility in the reporting period

Maternal mortality audit register

» » % of mothers and newborns receiving integrated postnatal care

Numerator: Number of mothers and newborns who received integrated postnatal care

PNC register / DHIS2

Denominator: Total number of deliveries at the facility in the reporting period

» » % of children under 10 who received integrated services in the same visit

Numerator: Number of children under 10 who received integrated services (IMCI assessment, immunisation check, nutrition screening, HIV status verification) in the same visit

Denominator: Total number of children under 10 seen at the facility in the reporting period

» » % of adolescents (10–20) accessing integrated adolescent-friendly services (AFHS)

Numerator: Number of adolescents (10–20 years) who accessed adolescent-friendly integrated health services (SRH, HIV, nutrition, mental health) at the facility

AFHS register / DHIS2

Denominator: Total number of adolescents (10–20 years) seen at the facility in the reporting period

» » % of under-5 deaths with a completed mortality audit

Numerator: Number of under-5 deaths with a completed mortality audit conducted as scheduled

Paediatric mortality audit register

Denominator: Total number of under-5 deaths recorded at the facility in the reporting period

» » % of notifiable disease reported

Numerator: Number of notifiable conditions reported

Denominator: Number of Notifiable conditions received

» » Emergency Care Mortality rates

Numerator: Number of Emergency care patients dead in 24 hrs

Denominator: Number of Emergency cases received

» Domain 3: Supply Chain and Medicines Management**» » % of commodities and supplies ordered on time**

Numerator: Number of commodities and supplies for which timely orders were submitted (via NMS CSSP+ or JMS) in the previous ordering cycle

Requisition records / NMS CSSP+ / LMIS

Denominator: Total number of commodities and supplies due for ordering in the reporting cycle

» » Average % availability for 50 tracer commodities (basket check)

Numerator: Number of tracer commodities (from the standard basket of 50) available at the facility at time of assessment

Physical stock check / bin cards

Denominator: 50 tracer commodities (standard basket as defined in the M&E Framework)

» » % of prescriptions made in accordance to the treatment guidelines

Numerator: Number of patients whose prescriptions were made in accordance with the standard treatment guidelines (UCG)

SPURS report / Support supervision visits

Denominator: Total number of patients given prescriptions reviewed in the reporting period

» » % of commodities with full accountability documentation

Numerator: Number of commodities with full accountability documentation (bin cards, stock counts, dispensing records reconciled)

Store records / bin cards

Denominator: Total number of commodities managed at the facility store

» » % of Adverse Drug Reactions (ADRs) reported as required**Numerator: Number of Adverse Drug Reactions (ADRs) reported as required through the pharmacovigilance system***Pharmacovigilance register / HMIS 105*

Denominator: Total number of ADR incidents identified/expected to be reported at the facility in the reporting period

» Domain 4:Health Management Information System (HMIS)**» » % of facility Units using digital HMIS tools****Numerator; Number of Units with digital HMIS tools***Facility inventory*

Denominator: Number of Units

» » % of client with updated information in Point-of-Care EMR/registers**Numerator: Number of client records/encounters entered on the integrated Point-of-Care EMR/register***EMR system / DHIS2 / EMR digitisation report*

Denominator: Total number of client encounters/records generated at the facility in the reporting period

» » % completeness of DHIS data reports**Numerator: Number of integrated HMIS forms submitted with complete data (≥90% fields filled, no missing critical fields)***DHIS2 / Data management report*

Denominator: Total number of integrated HMIS forms submitted by the facility in the reporting period

» » % of quartely data review and planning meetings conducted**Numerator: Number of quarterly integrated data review and planning meetings conducted at the facility***Meeting minutes / Data management report***Denominator: Total number of quarterly integrated data review and planning meetings scheduled at the facility****» Domain 5: Human Resources for Health****» » % Staffing levels****Numerator: Number of staff at facility***staff inventory/iHRIS and staffing norms***Denominator: Total number of staff approved for the facility****» » % of health workers deployed for integrated service delivery****Numerator: Number of health workers deployed/assigned to deliver integrated service delivery***Staffing / deployment records***Denominator: Total number of health workers at the facility****» » % of health workers trained on integrated service delivery****Numerator: Number of health workers trained on integrated service delivery***Training register***Denominator: Total number of health workers deployed at the facility****» » % of quarterly multidisciplinary team (MDT) meetings held****Numerator: Number of quarterly multidisciplinary team (MDT) meetings on integrated service delivery held***Meeting minutes / register*

Denominator: Total number of quarterly MDT meetings on integrated service delivery scheduled

» » % of quarterly HRH performance review meetings on ISD held

Numerator: Number of quarterly HRH performance review meetings on integrated service delivery held

HR meeting register / minutes

Denominator: Total number of quarterly HRH performance review meetings on integrated service delivery scheduled

» Domain 6: Laboratory Systems and Diagnostics

» » % of expected laboratory and diagnostic services available on the test menu

Numerator: Number of expected laboratory and diagnostic services available on the facility test menu at time of assessment

Physical assessment / CPHL standard test menu

Denominator: Total number of expected laboratory and diagnostic services defined in the Standard Test Menu for the facility level (Uganda CPHL standard)

» » Average turn around time for tracer tests

Numerator: Number of Health Facilities meeting the defined turnaround time (TAT) for tracer tests

Laboratory register / Support supervision visits

Denominator: Total number of Health Facilities assessed for turnaround time of tracer tests in the reporting period

» » % of tests results received through e-LIMS/EMR

Numerator: Number of Diagnostic test results received and documented through e-LIMS

e-LIMS system reports/EMR reports

Denominator: Total number of Diagnostic test results generated at the facility in the reporting period

» Domain 7: Community Systems and Routine Services**» » % of planned integrated community outreaches conducted**

Numerator: Number of planned integrated community outreaches (including home visits and campaigns) conducted by the facility

Outreach activity register

Denominator: Total number of integrated community outreaches planned by the facility in the reporting period

» » % of trained CHWs / VHTs available and active at this facility

Numerator: Number of trained CHWs/VHTs available and active (conducting community health activities) at the facility

VHT / CHW register

Denominator: Total number of CHWs/VHTs registered and attached to the facility

» » % of community referrals that are active in care at the facility

Numerator: Number of community referrals made to the facility confirmed to be active in care

Community referral register

Denominator: Total number of community referrals made to the facility in the reporting period

» » % of clients successfully followed up at community level

Numerator: Number of clients successfully followed up at community level (documented outcomes)

Community follow-up register

Denominator: Total number of clients requiring community-level follow-up in the reporting period

» Domain 8: Health Financing**» » % of facility budget allocated for integrated health services**

Numerator: Amount of facility budget allocated for integrated health services (disaggregated by funding source)

Budget / financial records

Denominator: Total facility budget for the reporting period

» » Budget absorption rate for integrated health services

Numerator: Amount of budget for integrated health services absorbed/spent in the reporting period

Financial records / PHC accounts

Denominator: Total budget allocated for integrated health services in the reporting period

» » % of quarterly PHC resource utilisation reviews conducted

Numerator: Number of quarterly PHC resource utilisation reviews conducted (with minutes and agenda item on integration)

Review register / minutes

Denominator: Total number of quarterly PHC resource utilisation reviews scheduled

» » Integration activities included in current facility work plan? (Yes / No / Partial)

Numerator: N/A — Binary/categorical indicator (Yes / No / Partial): Integration activities included in current facility work plan

Work plan review

Denominator: N/A — Binary/categorical indicator (Yes / No / Partial)

» Domain 9: Quality Improvement

» » % of multidisciplinary CQI meetings on Integration held

Numerator: Number of multidisciplinary CQI meetings on integrated service delivery held at the facility

Meeting minutes / CQI journal

Denominator: Total number of multidisciplinary CQI meetings on integrated service delivery scheduled

» » Number active CQI Projects on Integration

Numerator: Number of active CQI (QI) projects on integrated service delivery currently being implemented at the facility (with documented PDSA cycles in QI journal)

CQI journal / QI project register

Denominator: N/A (Count indicator — no denominator)

» » Client satisfaction rates for Integrated health services (Waiting time, stigma, privacy)

Numerator: Number of clients who rated integrated health services (waiting time, stigma, privacy) as satisfactory (≥80% score)

MoH Client Satisfaction System / SIMS survey

Denominator: Total number of clients assessed for satisfaction with integrated health services in the reporting period

Summary Notes (moved to another Tool)

1

Key Strengths Observed

Challenges Identified

Recommendations for Improvement

E. SHARED EXPERIENCES (moved to another Tool)

1

INSTRUCTION: Record shared experiences organized by standard. For each standard assessed, document what worked, what changed, the measured effect, and whether the change is replicable. This section feeds the quarterly learning briefs and change packages for the sentinel network

* Select a Domain

- ☐ Client Satisfaction ☐ D1: Leadership & Governance ☐ D2: Service Delivery ★ CORE
- ☐ D3: Commodity Management ☐ D4: Health Information Systems
- ☐ D5: Human Resources for Health ☐ D6: Laboratory Systems & Diagnostics
- ☐ D7: Community Systems ☐ D8: Health Financing
- ☐ D9: Quality of Care & Patient Safety

Visit / Date

yyyy-mm-dd

Intervention/Innovation

What Changed

Measured effect

Recommended learning for other facilities

C. SYNTHESIS & ACTION PLAN (moved to Another tool)

1

For each gap identified: specify the action, select the Intervention Type from the dropdown, name the Responsible Officer, and set the Timeline. Transfer all actions to the Action Tracker tab before leaving the facility.

*** Select a Domain**

- ☐ Client Satisfaction ☐ D1: Leadership & Governance ☐ D2: Service Delivery ★ CORE
- ☐ D3: Commodity Management ☐ D4: Health Information Systems
- ☐ D5: Human Resources for Health ☐ D6: Laboratory Systems & Diagnostics
- ☐ D7: Community Systems ☐ D8: Health Financing
- ☐ D9: Quality of Care & Patient Safety

Gap / Concern Identified

Action / Intervention

Intervention Type

- ☐ 1. CQI Intervention ☐ 2. Structural Intervention (e.g Hire Staff)
- ☐ 3. Facility Initiative ☐ 4. Other (Specify)

*** Other (Specify)**

Responsible Officer

Timeline
